

following questions:

- What is your job?
- Why do you do this job?
- What would happen if you stopped doing it?
- What do you want for the client?
- How old were you when you got this job?
- What is it like to do this job?
- Whom do you protect?
- What do you want the client to know?

When one interview feels complete, the client can go back to their original seat and describe the experience. When they are ready, or during the next therapy session, they can do the same for the opposing part. After these interviews, ask the client what they learned. How do they feel now toward the polarized parts?

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INTERVENING IN POLARIZATIONS

When firefighters are active, treatment providers tend to view them as the problem and ignore the internal criticism that managers dole out. In IFS, however, we map how each part affects other parts. Although it may take time for the client to map out the relevant pattern, they will recognize the behaviors and feelings they're naming as all too familiar.

When intervening with polarizing parts, we start by clarifying what drives the protectors (e.g., the need to engage in a distracting or soothing behavior versus the need to function and perform in various settings). When these parts are in overdrive, clients can feel confused and alarmed by their extreme rancor and impulsivity. In this regard, two tenets of the IFS model are immediately useful. First, constant conflict between protective parts is inevitable in an addictive system. Second, each team of parts, no matter how offensive, has positive intentions for the client.

For example, Santiago was a forty-two-year-old, Latinx, gay, cisgender high school teacher who first came to therapy for an overriding sense of depression, though he was also worried about his use of ecstasy and alcohol and his increasing impulsive sexual encounters that he considered risky. He had been in therapy for about six months. In this session, the therapist helped him identify his addiction polarity.

WORKING WITH A POLARITY



Santiago: I did it again this weekend. It's pretty crazy. I know I shouldn't go to apartments with strange guys. But I can't stop. When I get high, I don't care.

Therapist: Well, first, I appreciate your sharing this and being so honest. I'm know it's not easy.

Santiago: Yeah, you know I'd rather not discuss this. I mean it's fine to go out. I need to go out and do something besides read eleventh grade English papers. But last Saturday I got really messed up, and then I went with this guy—I guess to his place—which is really not cool of me to do. Why do I keep doing this? The guy was attractive but seemed kind of... a little too pleased with himself. Self-involved. And he drank a lot on top of the ecstasy, which affected his performance, I have to say.

Therapist: Can I reflect back what I'm hearing in parts language?

Santiago: Sure.

Therapist: First, I'm hearing those soothing, firefighter parts who want to get you out of your head. They like to get high, drink, and hook up. They don't care if it's risky for you to be out of it in a strange place with a guy you don't know. They don't think about risks. Is that right? (*Santiago nods.*) And then I'm hearing some other, more cautious parts. Your manager team worries about your safety. They track how often this happens, and they're concerned. Right?

Santiago: Yes. I mean, I'm worried about it. I'm getting fucked up, pardon my French, and it's not great to be out there that way. It's actually depressing, you know? Like, the next day I'm hungover, but also, just in general, I feel depressed. I'm still alone.

Santiago's manager team speaks out fully.

Therapist: So you're noticing that your mood is also affected. You already felt depressed, but after that night, it got even worse, and you have parts who feel even more alone now. (*Santiago nods.*) I wonder if you would try something with me?

Santiago: What?

Therapist: Visualize yourself at a table, maybe your kitchen table. You're seated at the head of the table. (*Santiago nods.*) On one side, you find all those parts who want to go out and get high—like really loaded—and hook up. Maybe you see them. Maybe you can just sense their energy.

Santiago: I see them standing by one side of the table. They're restless, like they can't sit down.

Therapist: Okay, that's fine. But ask them to stick around for just a bit.

Santiago: They'll stay, but I don't think they'll stay for long.

Therapist: Okay, that's fine. Invite the ones who worry about or feel critical of this behavior to sit on the other side of the table.

Santiago: There are lots of them.

Therapist: Make the table big enough for everyone, and you sit at the head. (*Santiago nods.*) And now just take a breath and see if they notice you. (*Santiago nods again.*) Let them know that you need both sides. That they're all important.

Santiago: The wild ones are skeptical, but they're here. They hear me.

Therapist: Great. And let those worried, critical parts know that they have an important point too.

Santiago: They're not sure they trust me either. But they could use some help on this, yeah.

Therapist: That's fine. Trust takes time! Ask them all to look at you for a minute. You're here to earn their trust and help them. We know they all mean well for you. There's room for everyone.

Santiago: They like the idea of not being alone. They don't really get where this is all going, but they're calmer.

Therapist: Great. Enjoy that calm with them. That's what can happen when you all get together with the intention of listening and learning. (*Santiago nods and sits quietly with his eyes closed.*)

The therapist focuses on facilitating this Self-to-part connection.



The therapist guided Santiago to notice and connect with both teams—the soothing firefighters and the worried managers. Although the concept was new to Santiago, and he didn't have any expectations about how it would help, he noticed that making a connection with both teams quickly calmed his parts, which was a reinforcing outcome within the session as well as for future Self-to-part connections.

DE-ESCALATING THE MANAGER-FIREFIGHTER POLARITY

As we've covered, manager and firefighter parts live in the same system and share the same desire to keep the system functioning. We don't choose between them; we build relationships with both teams. As critical managers feel understood and calm down, beleaguered firefighters get some relief. The reverse is also true. As firefighters tone down risky behavior, controlling and cautious managers get some relief too. Just as these two teams escalate together, they de-escalate together, and the sense of inner community builds. A client might say, for instance, "It feels good to understand this about myself" at the end of a session. Or they might validate a dominant manager by saying, "I feel sorry for my perfectionist. He never ever gets rest or appreciation." As the client develops warm, collaborative Self-to-part relationships inside, they also gain appreciation for the adaptive intentions of each protective team.

For example, Thea was a twenty-eight-year-old Greek American, partnered, heterosexual, cisgender woman who was currently in graduate school. She reported that she often drank large quantities of wine starting early in the day and also got high on cannabis. Recently, she missed a class she was supposed to teach because she was too intoxicated. Additionally, she was arguing with her boyfriend, Tom, frequently.

DE-ESCALATING A POLARITY



Therapist: I know it's not easy to talk about all this. It sounds like the drinking and getting high are really a worry for you right now, especially since you missed your class.

Thea's Firefighter: Yeah, I mean I've always partied plenty, which is fine. Who cares? But I really got in trouble with my graduate adviser for missing my class. That was pretty fucked up. And my boyfriend is on me now, too, which is pissing me off. Like he's not getting loaded? But I thought, okay, fine, I'll ease off for a while. And I was doing really good, ya know? Like even over the weekend. But then I got totally wasted.

She reports on a firefighter-manager polarity, speaking from both sides.

Therapist: What parts do